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This rating methodology updates and supersedes ICRA's earlier methodology document on this subject, published in September 2022. While this revised version incorporates a few modifications, ICRA's overall approach to rating hospitals remains materially similar.

Overview

This methodology document describes the key factors considered by ICRA in assessing the credit risk profile of hospitals. This methodology does not include an exhaustive treatment of all factors that are reflected in ratings but enables the reader to understand the rating considerations that are usually the most important.

The Indian healthcare industry comprises hospitals, diagnostic service providers, pharmaceutical companies, medical insurance, telemedicine, and medical equipment providers, among others. Hospitals comprise the largest component (60-70%) of the domestic healthcare industry, providing curative services, i.e., medical, nursing and related services. Some large hospitals also offer diagnostic services. The industry is fragmented, consisting of numerous players, ranging from large national and regional organised hospital players to unorganised standalone hospitals. While some hospitals are standalone ones, some are associated with medical colleges and other players also enter into operation and management (O&M) contract and franchisee agreements for expanding their presence in the domestic market.

Based on the range of services offered, hospitals can be identified as single speciality or multi-speciality hospitals. Moreover, based on the nature of services provided, the hospitals are classified as:

- Primary care – offer basic point-of-contact medical and preventive services, very few specialities
- Secondary care – includes general as well as speciality secondary care; five to 10 main clinical specialities including internal medicine, general surgery, gynaecology, paediatrics and orthopaedics; bed capacity typically ranges from 50 to 300 beds
- Tertiary care – highly specialised staff and technical equipment for handling complex treatments like cardiology, neurology, oncology, intensive care unit, high-risk trauma cases and specialised imaging units; clinical services highly differentiated by function; could be undertaking education and research as well; bed capacity range is typically upwards of 300 beds.

High real estate costs, equipment and operating costs, and shortage of medical professionals (leading to higher resource costs) result in elongated payback periods for hospitals. Some tend to have a star doctor model, which leads to revenue dependence on a specific set of doctors. In such cases, loss of key doctors can lead to decline in patient footfalls, which may adversely impact the overall revenues and profitability of a hospital. However, over a period of time, brand-based models

(backed by the established brand equity generated by the entity) have witnessed healthy growth in the domestic market.

ICRA's analysis focuses on the following key rating factors that are common for assigning ratings in the sector:

Industry risk assessment

- Growth prospects
- Competitive intensity
- Regulatory risks

Business risk assessment

- Scale and market position
- Operational profile
- Diversification
- Project risk

Financial risk assessment

- Profitability indicators
- Leverage and coverage indicators
- Working capital intensity
- Liquidity and adequacy of future cash flows
- Foreign currency risks
- Tenure mismatches and risks relating to interest rates and refinancing

Other considerations

- Parentage
- Financial flexibility
- Debt servicing track record
- Event risk
- Contingent liabilities / off-balance sheet exposures
- Consolidated financial analysis
- Accounting quality

Management quality assessment

- Quality of management and financial policies

Assessment of environmental, social and governance (ESG) risks

- Environmental (E) and social (S) risks
- Governance practices

Industry Risk Assessment

Growth prospects

The Indian hospital industry is underinvested with structural supply deficiencies compared to its counterparts in developed economies in terms of infrastructure and availability of clinical talent. Despite growth in both public and private sector investment in the healthcare sector in general and for setting up hospitals in particular, India continues to suffer from low availability of beds in relation to its population. The structural gap in demand and supply is one of the key growth drivers for this industry.

The Indian hospital industry has also witnessed increasing demand over the past few decades driven by rising penetration of medical services and health insurance. Growing population, increasing life expectancy, rising income, increasing incidence of non-communicable diseases related to lifestyles, greater awareness of personal health and hygiene, growing per capita spend on healthcare, increasing health insurance cover and increasing medical tourism are long-term positives for the sector. With increasing purchasing power, consumers are demanding and willing to pay for quality healthcare services. Over the years, share of revenues from medical tourism has also improved for large industry players given the relatively low cost of treatments in India than other countries. That said, medical tourism footfalls remain vulnerable to macroeconomic developments. Further, technological advancements and digital interventions on treatments (such as robotic surgeries) also support business prospects of industry participants.

Competitive intensity

Industry players range from a single doctor-run facility catering to limited specialities and low complexity procedures to large players catering to multiple specialities and highly complex procedures. Thus, the industry is fragmented, consisting of numerous players, ranging from large national and regional organised hospital players to standalone hospitals. Smaller players primarily cater to low complexity cases, which also requires relatively low capital investment, while larger players enjoy a dominant position in complex and high value treatments. Though standalone hospitals have limited resources with mostly only the local neighbourhood being aware about them, their relatively lower pricing helps them cater to a reasonable proportion of the population in each region. For large hospitals, in addition to brand equity of the hospital, the brand value of star doctors also tends to drive patient footfalls.

The capital investment required for setting up a multi-speciality hospital providing tertiary care is high due to requirement for procuring land and setting up a building in an easily accessible location and need for expensive medical equipment and infrastructure. In general, India remains under-penetrated in terms of hospital beds and the demand-supply situation favours the industry participants. However, given the high capex requirements, long gestation period for breakeven, and multitude of approvals required to set up a hospital, the barriers to entry in this segment are considerable, thereby, the competitive intensity is moderate.

Regulatory risks

The hospital industry operates in a regulated market, which includes number of approvals and licences from the Government and regulatory authorities and is governed by several state and Central Government laws, given its importance to the citizens and the economy. There are price caps on stents, knee implants, oncology drugs etc. and during the pandemic, price caps were introduced on Covid-19 treatments. While the impact of the regulatory intervention on the industry players has not been significant so far, more stringent or adverse regulations, if any, can impact the pricing power and thereby the profit margins of industry players. A price ceiling on specific essential or epidemic-related treatments in the interest of the public at large can also impact the revenue growth and profitability of industry players.

In addition to Government interventions, litigations, if any, could also pose a risk to the brand value of the hospital, impacting its footfalls and revenues. Litigations could pertain to patient(s) seeking a legal course of action against the hospital for providing medical treatment that falls short of the accepted medical standard of care. ICRA assesses the impact of these risks on an issuer's credit profile on best effort basis.

Business Risk Assessment

Scale and market position

ICRA views scale as an important parameter while assessing hospitals as it is a key driver of operating earnings (benefits of higher scale may accrue through efficiencies in purchasing and better absorption of fixed overheads). Scale is measured in terms of the revenues and the hospital network operated by any entity. A hospital with a diversified speciality profile is more likely to have a large revenue base which also lends stability to operations. Further, large hospitals which are mature assets tend to exhibit stronger earnings profile, providing necessary resources to expand and invest. Ability to consistently record revenue growth remains paramount for covering increasing inflationary pressure on operating costs and capital investments (equipment and real estate). Consistent revenue growth also indicates the hospital's ability to leverage its market standing and generate surplus earnings for capital investments, which are critical success factors. ICRA considers the low cyclicality associated with the hospital business favourably as demand for such services is largely insulated from macroeconomic cycles. A larger revenue base is viewed as a positive factor, as smaller hospitals tend to exhibit higher revenue volatility due to their dependence on few specialities or consultants, indicating high concentration risk.

The market position is reflective of the entity's scale, brand strength and track record of the hospital, and aids in capturing higher patient footfalls. A strong market position also determines a hospital's growth prospects and its ability to attract and retain patients and consultants, which lends stability to revenues and earnings to a large extent. The brand value of the clinical specialists (doctors) in the hospital also tends to have an impact on the market position of medium-sized hospitals. Some hospitals have been accredited by the National Accreditation Board for Hospitals and Healthcare Providers (NABH), under the purview of the Ministry of Commerce, Government of India, which provides guidelines and accreditation for running hospitals. This accreditation aids a hospital in empanelment with government institutions, corporates and other nations and helps attract more patients, both domestic as well as international. It also helps gain a better market position.

Operational profile

ICRA looks at various operational parameters which reflect a hospital's ability to optimise asset utilisation and generate stable earnings. These lend an insight into the management's business strategy and efficiency of operations. Managing brand equity and maintaining quality infrastructure to attract both medical consultants and patients and rendering effective patient services are critical success factors for efficient hospital operations. Further, ICRA also assesses the payor constitution for hospital entities. For measuring asset utilisation, ICRA assesses parameters such as occupancy levels, ARPOB, average length of stay (ALOS), growth in inpatient admissions and outpatient volumes. Hospitals generate income from the out-patient department (OPD), the in-patient department (IPD), pharmacy and laboratory (diagnostic) services. The hospital's revenue mix from the above segments (based on information availability) is also assessed for understanding the sustainability of growth and profitability levels.

Occupancy level: This refers to the proportion of beds used for in-patient treatments for a defined time period. This is calculated as total occupied bed days divided by the total operating bed days. Bed capacity indicates the number of beds which is set up by the hospital while the operating beds are the number of beds that a hospital can put into use at a specific point of time. Typically, mature hospitals are characterised by stable occupancies whereas new ones' witness improving occupancies. However, in some specialities, day-care procedures are prevalent with technological advancements, thereby in such cases, occupancy levels tend to be lower. That said, occupancy levels are assessed along with the patient footfalls, ALOS and ARPOB

of the hospital at any point of time. As occupancy levels improve to optimal levels, speciality mix improvements with increasing share of high-end procedures will ensure revenue stability and growth.

ARPOB: This refers to the average revenue generated by a hospital from every occupied bed day. ARPOB is generally a function of complexity of cases being handled, the criticality of the procedures/specialities in the hospital, pricing position driven by brand equity or doctor reputation, geographic location (tier-1 cities typically realise better prices than tier-2 and 3) and payor mix of the hospital. Annual price increases (to offset the inflationary pressures in cost of medical services) taken by the hospital players and periodic price increases granted by insurance companies support growth in ARPOB of hospital players to some extent. The pricing power is decided based on the location of any hospital and the competition in the said region. Presence of various standalone hospitals or those belonging to larger chains could pose competition risk and impact the ARPOB levels. Large hospital players tend to have Centres of Excellences (COE) to focus on key specialities like cardiac sciences, oncology, neurosciences, orthopaedics, and critical care, which generally contributes to the highest ARPOB compared to other hospitals. Higher the revenues from international patients and domestic cash/ insurance patients (compared to government scheme patients), the better the ARPOB levels. ICRA assesses ARPOB trends of issuers and does a comparative analysis with peers to understand an issuer's market positioning. In addition to ARPOB, ICRA also endeavours to assess the average revenue per patient, depending on information availability.

ALOS: This refers to the average time spent by a patient in a hospital. Lower ALOS helps in faster turnaround of beds, leading to higher number of patients being treated from existing bed capacities. The advancement in medical technology leading to increase in minimally invasive procedures and faster diagnosis and treatments, along with day-care procedures for critical specialities could result in lower ALOS.

Diversification

Diversity is measured in terms of geographic/ asset diversity, depth and breadth of services rendered and the diversity of the patient pool. A large and well-diversified hospital generally carries a strong brand perception and offers high-end value-added medical services, which augment profitability.

Geographic/ Asset diversification: Geographic/asset concentration risk is measured through the assessment of the number of hospitals operated across markets by the issuer and the degree of cash flow concentration from the various regions. Geographic diversification reduces the impact of regional regulations on the entity. However, high diversification with limited scale of operations could result in sub-optimal utilisation of resources and diffused management attention. For certain entities which have presence in only one or two locations but derive patient footfalls from various regions either by virtue of providing services at competitive rates or providing highly complex/critical services for a particular speciality, this risk is mitigated to a certain extent. ICRA undertakes assessment on this parameter on a best effort basis.

Speciality diversification: ICRA looks at the diversity of services offered in terms of the range of specialities and the complexity of procedures provided to the patients and notes the contribution of revenues from the major specialities to evaluate revenue concentration, if any, towards any particular speciality. A larger bouquet of specialities leads to improved scale and provides a larger scope for branding the hospital, which is factored in positively for the rating assessment. That said, high focus on a single speciality (more on chronic diseases), spread across geographical locations with a reputed doctor base and brand position, may not have a negative impact on the ratings. Further, complex medical service offerings improve a hospital's pricing power and lead to better margins.

Diversity of payor/ payor mix: ICRA looks at the diversity of the patient pool, such as international patients, domestic out-of-pocket paying patients, insurance, Government schemes (Central Government Health Scheme/employees health scheme), etc. Cash / insurance patients and international patients provide hospitals with better pricing power and low collection cycle, thereby reducing its working capital requirements. On the other hand, institutional patients that are Government scheme-linked or institution-linked tend to result in lower realisations and longer payment cycles. However, such payors may also

generate incremental volumes, which could provide a cushion in absorbing the high fixed costs of running a facility. A diverse patient pool would provide multiple levers to support revenues, enabling maintaining a base occupancy level and help mitigate concentration towards a particular segment.

Project risk

Given the growing demand for healthcare services, hospitals have been expanding by setting up new hospitals in a bid to diversify and garner a larger share of under-penetrated markets. This is in addition to the brownfield expansion via bed capacity additions undertaken at existing facilities. While analysing a hospital under construction, apart from the parent support (required to fund losses during the initial gestation period), ICRA analyses project-specific factors such as location, region-specific demand-supply dynamics, costs and sources of funding, and execution risks. The location and market determine the project's ability to attract patients (i.e., generate healthy occupancy and ARPOB), which dictates the potential to generate cash flows. The initial gestation period of any standalone hospital also depends upon its location, considering the flow of patient footfalls and presence of other hospitals in the same vicinity. ICRA assesses the investment required per bed against other comparable projects as well as existing hospitals and compares the same to expected earnings from the unit to assess the debt-servicing ability.

On account of long gestation periods, the mode of financing of the project influences the credit risk associated with the project (including tenure and interest rate related risks). The experience and ability of the management to ensure timely execution of projects within the budgeted cost levels play a key role in determining the viability and returns from the project. Expansion may also be done through operations & maintenance (O&M) agreements and leasing models for hospitals, using pay-per-use model for procurement of equipment etc. which are considered to be an asset-light route for growth. ICRA evaluates the same on a case-to-case basis and the potential revenues from the managed/ leased facilities are weighed along with the additional costs involved under this model, mainly in the form of payments to the asset owner.

Summary of the Salient Business Risk Factors

	Strongest		Weakest
Scale	>Rs. 3,500 crore	➡	Less than Rs. 100 crore
Geographic/Asset Diversification	Revenue share from single state <50%	➡	Revenue share from single state >85%
Average Revenue Per Occupied Bed (ARPOB)	ARPOB/day above Rs 50,000	➡	ARPOB/day less than Rs 15,000
Speciality Diversification	Top 2 specialities account for less than 30% of revenues	➡	Top 2 specialities account for more than 75% of revenues
Occupancy	Occupancy more than 75%	➡	Occupancy less than 35%
Payor Mix	Cash + Insurance + International patients account for more than 80% of revenues	➡	Cash + Insurance + International patients account for less than 50% of revenues

Financial Risk Assessment

The various financial metrics assessed by ICRA could be divided into four categories, viz. profitability, leverage, coverage and liquidity. This section provides a brief summary of why ICRA considers these ratios to be important. For a more detailed description, readers may refer to the note titled, *Approach for Financial Ratio Analysis* published on ICRA's website. Some of the key metrics analysed are discussed below.

ICRA also draws up projections on the likely financial position of the company based on the expected movements in operating performance factoring in capex and investment requirements as well as upcoming debt obligations to study the impact on revenue growth and profitability, cash flows, leverage as well as debt protection indicators. Depending on the uncertainty around how the various credit drivers could evolve in the future, ICRA also carries out sensitivity analysis to assess the impact of the key variables on the various financial metrics. In case of groups consisting of entities with strong financial and operational linkages, various parameters such as capital structure, debt coverage indicators and future funding requirements are assessed at the consolidated/group level.

Profitability metrics

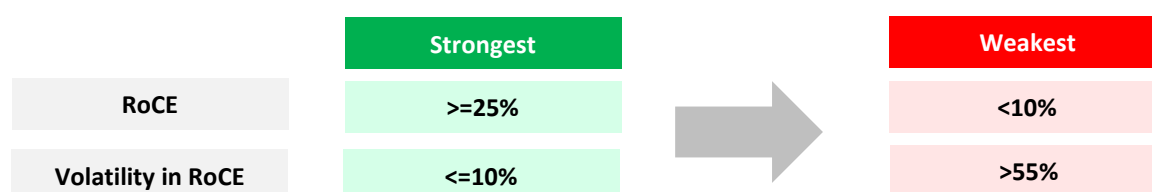
Hospitals have high operating leverage due to significant fixed costs in the operating structure. Medical consumable costs and employee costs (including payouts to doctors/medical consultants) comprise a sizeable portion of a hospital's operating costs. While hospitals employ doctors on the payroll with fixed remuneration structure, some key medical consultants typically tend to have revenue share along with a minimum monthly guaranteed payment. A variable remuneration structure makes the interest of the key consultants better aligned with that of the entity and it also reduces cost pressure in case of decline in revenues/volumes. A large revenue base leads to economies of scale in terms of cost efficiencies in purchasing medical consumables which in turn supports operating profit margins (OPM). In addition, the OPM is also a function of optimum utilisation of operating beds and pricing power of the hospital (which also depend upon competitive intensity and demand scenario of the region). Further, some hospital players incur rental costs (pay-per-use model or fixed rentals) on large medical equipment in lieu of purchasing the same, to reduce the capital investment in the hospital, which may impact the OPM in the early stages of operations; however, this reduces the fixed capital costs, while providing flexibility in managing the cost structure. Newly-built facilities take time to ramp up and face pressure on profitability during the initial years of operations,

due to high unabsorbed fixed costs. On the other hand, a mature facility with a track record of operations will have and moderately growing revenue profile. A hospital's performance is viewed in tandem with its vintage.

ICRA uses OPBDITA as an important metric to measure profits for the hospital industry and also evaluates stability in the OPM over a period of time, as it measures the entity's ability to withstand competitive pressure apart from event-specific risks. Also, ICRA evaluates RoCE since it is a measure how efficiently an entity sweats/ utilises its assets and since an entity's ability to consistently generate the RoCE over and above its cost of capital reflects well on its long-term business viability. However, the RoCE of an entity may slide in a given year if it undertakes a large capital expenditure (capex) programme or pursues an acquisition. ICRA does not necessarily perceive this as a negative, if in its assessment, the capex or the acquisition, is likely to generate adequate returns in the future.

Validation of Business Risk through Profitability Metrics

[Indicative Metrics¹]



Leverage and coverage indicators

ICRA's assessment of the financial risk profile of the entity hinges on its ability to generate healthy cash flows to reinvest in the business as well as meet the debt servicing obligations. The financial policies - past as well as future - are a key rating factor to ascertain the risk appetite of the management and the impact of the same on the entity's financial performance.

Hospitals have significant re-investment requirements for expansion and upgradation of facilities. ICRA places considerable emphasis on measuring the adequacy of cash flows (after meeting operating and investment needs) to meet debt repayment obligations. The debt profile of hospitals is typically skewed towards long-term debt on account of capital intensity, with the cash and carry nature of the business limiting working capital requirements. ICRA analyses the indebtedness, including features such as interest rate, tenure, and structure of the debt drawn/ to be drawn.

Leverage ratios are an indicator of the degree of financial flexibility an entity enjoys in terms of its ability to raise funds from alternative sources in times of financial distress. Such flexibility is reflected in an entity's Total Debt-to-OPBDITA multiple. A low leverage ratio indicates better ability to withstand volatility in cash flow generation during situations of economic downturn, competitive challenges, unexpected costs or regulatory changes. As a result, it can help the entity to continue to invest in new technologies, capex and entry in new markets during adverse conditions.

¹ The indicative financial metrics mentioned here and elsewhere in the document are intended to provide a broad overview to the readers regarding what ICRA generally considers as 'relatively strong' or 'relatively weak' metrics. It is, however, possible that an entity has relatively weaker metrics on one or more financial parameters, but its credit risk is assessed to be low because of other mitigating factors, including (but not limited to) stronger metrics on other financial parameters, a healthy business risk profile, strong financial flexibility or a strong promoter group that is willing to extend distress support to it.

Assessment of Leverage

[Indicative Metrics]

	Strongest	Weakest
Indebtedness Ratio	$\leq 0.9x$	$> 3.0x$
Debt to Profit Ratio	$\leq 0.5x$	$> 5.0x$

Coverage is a measure of an entity's debt-servicing ability and is calculated as the ratio of profits to the debt servicing obligations in a given time period. The higher the ratio, the higher the cushion available with an entity to withstand variability in profits for making good its financial obligations. Coverage is a function of an entity's profits, leverage and debt characteristics (in terms of cost of debt and repayment schedule). The interest coverage indicator reflects the company's ability to fund the cost of external borrowings after meeting all the operating expenditure requirements. The debt service coverage ratio (DSCR) is a measure of an entity's debt-servicing ability and is calculated as the ratio of profits to the debt servicing obligations in a given time period. Entities with higher profitability and lower leverage will generally have better coverage ratios and thereby healthier financial risk profiles. ICRA is particularly concerned with an entity's capability to honour its contractual obligations under stress conditions. The more robust an entities performance under stress scenarios, the better it is from a credit evaluation perspective.

Assessment of Coverage

[Indicative Metrics]

	Strongest	Weakest
Interest Coverage	$\geq 18.0x$	$< 2.0x$
DSCR	$\geq 4.0x$	$< 1.1x$

Working capital intensity

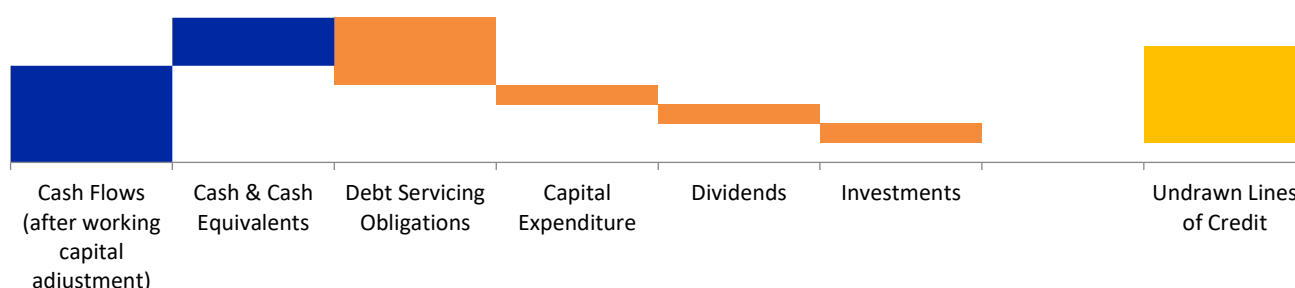
Generally, the working capital intensity of the entity depends on the payor mix of the hospital. While cash/walk-in patients provide immediate cash realisations, insurance patients tend to have a payment cycle of ~30-45 days (mainly for completion of documentation formalities from the third-party administrator (TPA)). Central Government, state governments, armed forces and PSUs operate various schemes for their employees (along with their dependents) and empanelment with these public sector schemes enables a hospital in attaining reasonable volumes. Sans the empanelment, a hospital misses out on business from large number of beneficiaries enrolled under these schemes. However, the payment cycle of these public-sector schemes is long and in some instances in the past has become significantly stretched, which can lead to cash flow mismatches, high working capital intensity of operations and in some cases, stretched liquidity position. ICRA compares the working capital cycle of the entity with its peers to gauge the rated entity's working capital management system as a proxy of their business model and efficiency.

Liquidity profile

Liquidity is the measure of an entity's ability to meet its short-term cash obligations from various internal or external resources. Internal resources include cash flows from operations, unencumbered cash and cash equivalents on balance sheet and cash inflows expected from the monetisation of physical and financial assets. External resources include undrawn lines of credit or equity capital. The short-term obligations include both committed as well as the contingent claims on an entity's cash, including the debt servicing obligations, working capital requirements, capital expenditure and other investment outlays, dividend and

share buyback-related outflows, besides the sudden demand arising from the crystallisation of discrete events such as unfavourable outcome of an ongoing litigation. The higher the cushion available between the resources available (especially internal resources) and the obligations, better the liquidity profile of an entity. Liquidity is generally assessed in conjunction with the vulnerability of an entity to timely refinance/renewal of short-term sources of funding. Depending upon the circumstances, an entity that has a relatively modest liquidity profile but a strong refinancing ability may not be viewed too unfavourably. ICRA also notes that the liquidity available with an entity may be for a temporary period and hence an entity's overall policy towards maintaining adequate liquidity (given the trade-off between returns and liquidity) is accorded due importance in the analytical approach².

Liquidity snapshot over any defined period



Cash flows

The rating exercise is primarily focused on assessing the future debt-servicing capability of an entity. Since it is cash that is required to service the debt obligations, it is imperative that a cash flow analysis is undertaken to evaluate the external funding requirements and likely financial position of the company, going forward. A cash flow statement represents the sources from which cash is generated and its deployment. Analysed here are the trends in an entity's fund flow from operations, cash consumed to fund the working capital, the retained cash flows after paying out dividends or carrying out share buybacks, and the free cash flows after meeting debt repayment obligations and capital expenditure needs. ICRA also looks at the funding requirements of a company and the funding options available to it.

Capital expenditure and investment plans

The capital expenditure plans of a hospital entity reflect its plans for capacity expansion, geographic diversification, and upgradation/maintenance of its medical equipment etc. The quantum of capital expenditure and funding plans for the same are also evaluated to understand their impact on the financial risk profile of the entity. That said, ICRA notes that a lot of hospital entities are now opting for the pay-per-use models to reduce the upfront cash outflow towards equipment purchase.

² For more details on how ICRA assesses liquidity, readers may refer to the document titled, "Liquidity Analysis of Entities in the Non-Financial Sector" published on ICRA's website.

Foreign currency risks

Foreign currency risks for the industry primarily arise on account of the import of equipment, operations outside India and foreign currency denominated debt. While assessing the exposure of an issuer to foreign currency risks, ICRA focusses on the impact of adverse movement in foreign exchange rates on the cost structures, profits and net cash outflows, besides evaluating the hedging mechanisms in place. This apart, currency movements impact demand for medical tourism.

Tenure mismatches and risks relating to interest rates and refinancing

The hospital industry remains highly dependent on the banking system to meet its funding requirements, with limited access to capital markets except for a few large corporate entities. Large dependence on short-term borrowings to fund long-term investments or other long-term funding requirements can expose an entity to significant re-financing risks, especially during periods of tight systemic liquidity. ICRA evaluates the extent of such mismatches and the mitigating factors therein. One source of mitigation could be the existence of adequate buffers of liquid assets/committed bank lines to meet short-term obligations. Another source of mitigation could be the entity's strong financial flexibility to be able to garner fresh funds at a short notice or a potent ability to refinance. Further, ICRA evaluates the extent to which an entity might be impacted by movement in interest rates.

Other elements of credit risk assessment

Parentage/group support

While the credit rating of an entity is a function of its standalone credit profile, in certain cases, the entity's credit quality can also be driven by the relationship with its parent or the promoter group (henceforth referred to as the parent). If the parent's credit profile is relatively strong than the rated entity, ICRA assesses the ability and the likelihood of the parent extending extraordinary support to the entity. Support here means financial support from the parent expected to be available to the entity in the form of loans, equity, extended credit period, advances etc. in times of credit or liquidity stress on the entity. Support here does not mean operational support in the form of new business opportunities, technology sharing, distribution network sharing and so on as these aspects are factored in the standalone credit profile assessment itself. It may be noted that promoters in their individual capacity, or private equity firms/ other financial investors are generally not treated as parents for assessing the likelihood of extraordinary financial support coming in. If the parent's credit profile is weaker than the rated entity, the entity's rating may be lower than what its standalone credit profile assessment would have merited, given the possibility that the entity may at some point of time be bound to extend financial support to its weaker parent, possibly to the detriment of its own credit profile³.

Financial flexibility

An entity's financial flexibility (or the lack thereof) is reflected in its ability to access the capital or the money markets at short notice, attract diverse and marquee investors and enjoy the confidence of banks, financial institutions and intermediaries. A strong financial flexibility allows an entity to raise fresh borrowings or refinance existing ones in quick time, whenever required. Financial flexibility could depend on factors such as an entity's large scale of operations with strong financials, large, unencumbered cash flows, unencumbered assets and the flexibility to borrow against such assets, or strong parentage or linkages with a strong group. In contrast, among the various measures of an entity's depleting financial flexibility, one relates to a high share of pledged promoter shareholding. A sign such as this may imply that the entity might be persuaded to distribute high dividends or support the promoter group through other means to the detriment of its own credit profile. If the promoters fail to repay their loans (availed by pledging of shares) or top up collateral when required, the lenders could sell the

³ For more details on this, readers may refer to the document titled, "Rating Approach—Implicit Parent or Group Support", published on ICRA's website.

pledged shares. In some cases, this could trigger a change-of-control clause in the rated entity's bond indentures or loan documents and require it to redeem its debt ahead of schedule, creating a liquidity squeeze, besides affecting fresh capital raising ability. Financial flexibility could also be impacted in cases of adverse industry developments, weakening business profile, or management and governance concerns, which could translate into a sharp decline in market capitalisation or spike in bond yields and consequently constrain an entity's ability to raise fresh capital or materially increase its cost of capital.

Debt servicing track record

The debt servicing track record of the company forms an important rating consideration. Any history of past delays or defaults in meeting interest and principal repayment obligations reduces the comfort level with respect to the company's future debt servicing capability and willingness. Nevertheless, the reason behind past defaults is also analysed, which could also be due to adverse demand situations in the underlying segments.

Event risk

ICRA recognises the possibility of events such as unrelated diversification, mergers and acquisitions, business restructuring, asset sales and spin-offs, litigations, equity infusion and refinancing, which could have a material impact on the credit profile of an entity. Incorporating the impact of such discrete events in the credit rating, from the beginning, is often difficult. To make rating decisions in such cases, ICRA applies its analytical judgment based on the rated entity's track record, the credibility of the management and the experience of having seen similar situations play out in other entities. However, given the nature of such events, it is possible that the rating may undergo a material change later, upon the occurrence of the event.

Contingent liabilities/ off-balance sheet exposures

The likelihood of devolvement of contingent liabilities/off-balance sheet exposures and the financial implications of the same are evaluated.

Consolidated financial analysis

Some hospital entities also acquire shareholding in other entities to support their growth and geographic diversification. Such transactions typically entail acquisition of partial shareholding in another hospital entity, which is owned by a doctor promoter who is expected to continue to be a part of the operations.

While evaluating the financial risk profiles of companies that have subsidiaries and associate companies, ICRA analyses consolidated/group level financial indicators in terms of capital structure, debt coverage indicators and future funding requirements. For more information on the approach, readers may refer to the methodology titled *Rating Approach – Consolidation* available on ICRA's website.

Accounting quality

ICRA reviews the accounting policies, notes to accounts, auditors' comments and other disclosures that are parts of the Annual Report of a rated entity. Deviations, if any, from the accounting standards/practices are assessed and the financial statements of the entity are adjusted to reflect the impact of such deviations.

Management Quality Assessment

In addition to the industry, business and financial risk analysis, all credit ratings incorporate an assessment of the quality of the rated entity's management and its financial policies.

Quality of management and financial policies

As a part of its process, ICRA undertakes discussions with the rated entity's management to understand its views on past performance as well as its future plans and strategies, besides the outlook on the industry. Some of the points assessed are:

- Experience of the promoter/management in the industry
- Commitment of the promoter/management to the rated entity
- Risk appetite of the promoter/management and risk mitigation plans
- Policies on leveraging, managing interest rate and currency risks
- Management's past success in introducing new projects and managing changes in the external environment
- Management's plans on new projects, acquisitions and expansions
- Track record of balancing the interests of shareholders, creditors and other stakeholders

Periodic interactions with the management help ascertain the shifts, if any, in their financial policies.

Assessment of Environmental, Social and Governance (ESG) Risks

The assessment of ESG risks by ICRA involves a broad range of considerations that pertain to the sustainability of an entity with a focus on aspects that can have a material impact on its credit quality. While the E&S risks tend to be both sector-related as well as entity-specific and could be driven by external factors such as regulations or demographic changes, the G risks are largely entity-driven. The impact of the E&S risks on an entity's credit profile tends to be asymmetric. If the ESG risks are material but unmitigated, these generally translate into pulling down the rating, but generally, the ratings are not pushed up even when the ESG context is favourable.

Environmental (E) and social (S) risks

As this methodology highlights, while undertaking credit assessment of entities, ICRA seeks to incorporate all relevant credit considerations into its rating decisions while taking a forward-looking view on the risks and the mitigants. The relevant credit considerations include (sometimes overtly, sometimes covertly) the E&S factors that could affect the rated entity/transaction. While ICRA's analytical approach does not explicitly disaggregate these risks to assess their impact on the rating, the risks are often assessed broadly. Further, it is not always feasible to fully or precisely disaggregate the sub-components of E&S risks in credit analysis since these considerations often tend to overlap.

That said, the materiality of the E&S risks and the time horizon over which they are expected to crystallise differs widely across sectors and entities. In some cases, while the E&S risks could be material but their effect on the credit profile may be muted because of other fundamental strengths of the entity. In other cases, the adverse impact of the E&S risks is expected to play out in the distant future, and hence these considerations do not necessarily weigh on the rating today—with the expectation that when these risks manifest in the distant future, the rated entity by then would possibly adapt itself by realigning its business model.

While evaluating the E&S risks, ICRA's objective is only to assess the direct and indirect ones that an entity faces and how it already is or is intending to mitigate the impact of such risks on its credit profile. As an example, ICRA only assesses whether an entity is exposed to physical climate risks, or carbon transition risks such as those arising from changes in regulations or other environmental and social risks; and seeks to understand the various mitigation and adaptation approaches that the

entity is implementing to mollify these risks.

The hospital industry does not face any major physical climate risk. However, hospitals need to comply with environmental laws and regulations pertaining to handling and disposal of bio-medical specimens, wastewater, infectious and hazardous waste. This requires investments in infrastructure to handle the generated waste, treating the wastewater effluents and conserve energy. Accordingly, entities in the industry have a moderate exposure to environmental risks.

Exposure to social risks is also moderate for hospitals. Social risks for industry players include litigation exposure, and compliance standard requirements given the importance of the service being provided. Industry players also face risks relating to their ability to retain qualified clinical specialists such as doctors and nurses etc. Further, regulatory interventions such as price control measures, imposition of restrictions, if any, specifically levied could impact the earnings of industry players.

Governance practices

A sound corporate governance structure attempts to make clear the distinction of power and responsibilities between the Board of Directors and the management. The composition and conduct of the Board, besides the entity's adherence to legal and statutory compliance requirements, are factored in during credit assessments. ICRA seeks to gain a qualitative understanding of an entity's commitment to following transparent and credible practices by the way its financial statements are reported, their level of disclosures, consistency in communication and the openness about sharing information during the credit rating exercise. Besides, the corporate group structure (whether simple or complex), the rated entity's related party transactions and instances of supporting group entities at the expense of debt holders are also assessed.

Summing Up

ICRA's credit ratings are a symbolic representation of its opinion on the relative credit risk associated with the instrument being rated. This opinion is arrived at following a detailed evaluation of the entity's business and financial risks, its competitive strengths, its likely cash flows over the life of the instrument being rated, and the adequacy of such cash flows vis-a-vis its debt servicing obligations. ICRA's approach to rating hospital entities also incorporates an assessment of the company's market position, scale, operational performance, diversification level, project risk, capex plans.

ANNEXURE

Summary of rating factors and an example to illustrate the key building blocks of a credit rating

		Strong			Comfortable			Adequate			Moderate			Weak		
Industry Risk	Industry Position															
	Scale															
Business Risk	Geographic Diversification															
	Average Revenue Per Occupied Bed															
	Speciality Diversification															
	Occupancy															
	Payer Mix															
Financial Risk	Profitability and Earnings Stability															
	Leverage															
	Coverage															
		Enhance					Support/ Neutral					Hinder				
Do these factors enhance or hinder the credit profile?	Diversification															
	Refinancing Dependence, Liquidity and Financial Flexibility															
	Currency Risk															
	Financial Policy															
	Management, Governance & Reporting															
		Very High					High			Moderate				Low		
Parent Support	Likelihood of Parent Support															
	Rating of Parent	AAA	AA+	AA	AA-	A+	A	A-	BBB+	BBB	BBB-	BB+	BB	BB-	B/ C category	
	Final Rating	AAA	AA+	AA	AA-	A+	A	A-	BBB+	BBB	BBB-	BB+	BB	BB-	B/ C category	

The above graphic is only for illustration purpose and does not represent a rating output from a formulaic model. The ratings assigned by ICRA are determined by Rating Committees based on both quantitative and qualitative considerations.

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